



ENT Consultation Note – Initial Cervical Lymphadenopathy Assessment

Service: Otolaryngology – Head & Neck Surgery
Location: NorthStar ENT Clinic, Outpatient Pavilion
Encounter Type: In-person consultation
Date of Service: 2021-10-12
Referring Provider: Primary Care (external)

****Chief Concern:**** Persistent cervical lymphadenopathy (LAN) noted by patient on self-exam; request for diagnostic clarification.

****History of Present Illness:**** The patient reports painless enlargement of lymph nodes in the upper anterior neck over approximately several months, without associated odynophagia, dysphonia, dental infection, sinus symptoms, or recent upper respiratory illness. He denies constitutional symptoms (fever, drenching night sweats, unintentional weight loss), new rashes, arthralgias, or chronic diarrhea. He reports no known TB exposure and has no history of smoking. He is concerned about the duration of LAN and requests a structured diagnostic plan.

****Past History / Medications / Allergies:**** At this stage, limited external records are available. The patient reports no known drug allergies. He is not taking regular prescription medications. Immunization history and travel history are not fully captured at this visit and will be clarified on follow-up.

****Physical Examination:**** Well appearing and in no distress. Oropharynx without lesions. No tonsillar asymmetry. Neck: palpable mobile lymph nodes in the upper anterior cervical chain; non-tender, not rock hard, without overlying skin changes. No palpable thyroid masses. Cardiopulmonary exam unremarkable by referring note. No hepatosplenomegaly appreciated on brief exam.

****Assessment:**** Persistent cervical LAN—etiology unclear. Features are not overtly suspicious on exam, but duration warrants imaging and tissue sampling for reassurance.

****Plan:**** (1) Ultrasound neck with attention to level II nodes; (2) Ultrasound-guided FNA of dominant node with cytology and flow cytometry; (3) Baseline labs (CBC, LDH, ESR/CRP) to be coordinated with primary care; (4) Return visit after results to review next steps. Patient advised to seek care promptly for new B-symptoms or rapid growth.



Addendum / Comparison and Administrative Notes

Comparison statements reflect the most recent prior studies available in the network at the time of reporting. If external imaging exists, formal comparison may be limited until images are imported. Clinicians are encouraged to interpret findings in the context of the patient's broader longitudinal record and to correlate with physical examination and symptoms.

Quality Assurance: This report was auto-formatted from a structured radiology/clinic template and reviewed for internal consistency. Minor variations in measurements may occur due to technique, probe position, and interpreter variability.